



# Review

## Emotions and client participation in jurisdictional contestation

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| Abstract:                       | <p>We develop a model of how emotions shape the participation of professions' clients in episodes of jurisdictional contestation. Our model begins with a framing contest between a social movement that disrupts a profession's jurisdictional control and the profession that defends it. We theorize how, through adversarial framing efforts, the movement and profession each seek to evoke emotions in particular ways to shape the actions of clients in their favor. We then explore how the emotional resonance of this framing contest leads individual clients to support, to varying degrees, one or both contestants. We argue that clients experiencing different configurations of pride, anger, shame, and fear—or ambivalence when these emotions overlap in conflicting ways—enact one of five modes of participation. With this article, we contribute to the literature on professions by conceptualizing client participation in jurisdictional contestation across analytical levels; considering the role of a constellation of intertwined social emotions in this process; and introducing a typology of five modes of client participation in jurisdictional contestation. We develop our model by drawing on empirical examples from health-related professions, but we also discuss its generalizability to other work domains and stakeholders.</p> |

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**EMOTIONS AND CLIENT PARTICIPATION  
IN JURISDICTIONAL CONTESTATION**

**ABSTRACT**

We develop a model of how emotions shape the participation of professions’ clients in episodes of jurisdictional contestation. Our model begins with a framing contest between a social movement that disrupts a profession’s jurisdictional control and the profession that defends it. We theorize how, through adversarial framing efforts, the movement and profession each seek to evoke emotions in particular ways to shape the actions of clients in their favor. We then explore how the emotional resonance of this framing contest leads individual clients to support, to varying degrees, one or both contestants. We argue that clients experiencing different configurations of pride, anger, shame, and fear—or ambivalence when these emotions overlap in conflicting ways—enact one of five modes of participation. With this article, we contribute to the literature on professions by conceptualizing client participation in jurisdictional contestation across analytical levels; considering the role of a constellation of intertwined social emotions in this process; and introducing a typology of five modes of client participation in jurisdictional contestation. We develop our model by drawing on empirical examples from health-related professions, but we also discuss its generalizability to other work domains and stakeholders.

## INTRODUCTION

Professions are deeply implicated in the maintenance of established societal institutions through their ongoing jurisdictional control, which refers to their members' monopolistic autonomy over their work (Freidson, 1989; Scott, 2008). Over three decades of research inspired by Abbott's (1988) *System of Professions* has focused on how inter-profession struggles for this control—jurisdictional competition—shapes continuity and change in work domains (Langley, Lindberg, Mørk, Nicolini, Raviola & Walters, 2019). In addition to rival professions, other types of collective actors can shape continuity and change by challenging a profession's jurisdictional control and engaging in struggles—jurisdictional contestation—over the boundaries and content of professionals' work (Ackroyd, 1996). Such contestation is well documented and theorized in organizational studies of how professionals are regulated (e.g., Suddaby, Cooper and Greenwood, 2007) and managed (e.g., Kirkpatrick, Ackroyd and Walker, 2005). Challenges to a profession's jurisdictional control from types of actors other than regulators and managers of organizations in which professionals work have received much less attention, however. In our polarized contemporary times where professions and other societal institutions are intensely contested by social movements (Gurri, 2018), a shift in focus towards these actors is increasingly needed (Eyal, 2019). Our paper addresses this gap.

We draw upon research adopting the relational perspective on professions, which focuses on how professions seek the support of different audiences to maintain their jurisdictional control (Anteby, Chan, & DiBenigno, 2016; Eyal, 2013). It highlights that, to defend its jurisdictional control from contestation, a profession needs to secure and preserve the trust of various audiences in its expert knowledge (Muzio, Aulakh, & Kirkpatrick, 2020; Anteby, et al., 2016; Evetts, 2006). Because the importance of clients as such an audience is increasingly noted (e.g., Chan & Hedden, in-press; Huising, 2021; Eyal, 2019), we focus on constestation of a

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profession’s jurisdiction driven by clients’ mistrust in the profession’s expert knowledge. To do so, we use Fligstein and McAdam’s (2012) conflict-oriented theory of fields. Specifically, we theorize an episode of jurisdictional contestation as a framing contest (Kaplan, 2008) between two collective actors in which a client-led social movement acts as challenger, and a profession as incumbent. The social movement has emerged from a marginalized community seeking emancipation from dependency on the profession whose expert knowledge constructs community members as flawed. Based on the experiential knowledge collectively held by its adherents, the social movement disrupts the profession’s jurisdictional control, while the profession tries to preserve trust in its expert knowledge. These collective actors engage in adversarial framing efforts to promote their opposing institutional projects to individual clients of the profession.

Because published research and first-hand accounts describe episodes of jurisdictional contestation as deeply emotional (Taylor, 2000; Bayer, 1987), we use literature on how emotions shape people’s participation in contested social arrangements (Voronov & Weber, 2020; Zietsma, Toubiana, Voronov & Roberts, 2019; Jasper, 2011). We put a specific constellation of four emotions—pride, anger, shame, and fear (as well as ambivalence when these emotions overlap in conflicting ways)—at the core of our theorizing because the emancipation of members of marginalized identity movements is theorized as an emotional process in which participants move through experiences of fear and anger to convert their shame of individual deviance into a pride of collective affirmation (Britt and Heise, 2000). Further, this specific constellation of emotions is described in many studies of social movements (e.g., Gould, 2009; Schrock et al., 2004; Bayer, 1987). Building on these ideas, our model explains how, through their adversarial

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3 framing efforts, a social movement and a profession each seek to evoke these specific emotions  
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5 in particular ways to shape the actions of clients in their favor.  
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8       Efforts to evoke specific emotions do not always achieve intended effects, however. We  
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10 therefore draw upon the literature on emotional resonance, which highlights how felt emotions  
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12 shape a person's participation in social life (Giorgi, 2017; Robnett, 2004). We apply these  
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14 insights to the context of a profession whose jurisdictional control is being disrupted by a social  
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16 movement. Specifically, we argue that different emotional experiences —i.e., pride, anger,  
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18 shame, and fear, as well as the ambivalence felt when these emotions overlap in conflicting  
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20 ways—lead individual clients to support one or both framing contestants, to varying degrees, in  
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22 specific ways. We theorize five modes of client participation in jurisdictional contestation, which  
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24 we term escape, opposition, accommodation, acquiescence, and stewardship, and describe how a  
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26 client enacting each mode seeks to shape established arrangements towards change or continuity.  
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28 We develop our model by drawing on empirical examples from health-related professions, which  
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30 have faced numerous historical and recent episodes of jurisdictional contestation; but we also  
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32 discuss its generalizability to other work domains and other stakeholders.  
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38       With our model, we make several contributions to the literature on professions. First, by  
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40 theorizing the participation of individual clients in an episode of jurisdictional contestation as  
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42 resulting from a framing contest between a challenger social movement and an incumbent  
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44 profession, our model highlights that the authoritative nature of a profession's expert knowledge  
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46 can be countered by the experiential knowledge collectively held by movement adherents. In this  
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48 way, we complement existing research on jurisdictional contestation, which tends to focus on  
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50 regulators and managers as the main challengers. Whereas these actors use formal authority to  
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52 challenge a profession and redefine the boundaries and content of its work (e.g., Waring &  
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Currie, 2009), our model shows how a social movement can use an alternative epistemic basis to draw individual clients into a collective challenge to a profession’s jurisdiction.

Second, we consider the role of multiple, intertwined social emotions in this process. Rather than focusing on a singular emotion (e.g., Gill & Burrow, 2018), we focus on pride, anger, shame, and fear as a constellation and illustrate how each side in the movement-profession framing contest seeks to evoke these emotions in individual clients, but in different ways and to different ends. Finally, we introduce a typology of five modes of client participation in jurisdictional contestation that highlights how specific configurations of emotional resonance lead an individual client exposed to a framing contest to enact a particular mode. By doing so, we bridge the collective and individual levels of analysis while laying the foundation for future research into synergies and tensions between clientele segments enacting different modes.

**PROFESSIONS, CLIENTS, AND EMOTIONS**

A jurisdiction “is the link between a profession and its work” (Abbott, 1988: 20). A profession controls a jurisdiction when it holds exclusive rights to interpret and respond to the needs of its clients in a given work domain (Scott, 2008; Hughes, 1958). Abundant research has focused on inter-profession competition for control over work domains (Langley et al., 2019). But collective actors other than rival professions can also challenge a profession’s jurisdictional control and engage in struggles over the boundaries and content of professionals’ work. Most research on these struggles, termed jurisdictional contestation, focuses on regulators of professions or managers of organizations in which professionals work (e.g., Currie & Suhomlinova, 2006; Kellogg, 2009). Indeed, the negotiated nature of the regulatory bargain between states and professions to define legal boundaries around the work over which a

profession's members are granted jurisdictional control has long been at the core of sociological research on professions (Abbott, 1988; Suddaby et al., 2007).

In addition to regulators, organizational studies of professions have focused extensively on managers as key contestants of professionals' jurisdictional control (e.g., Courpasson, 2000; Kirkpatrick et al., 2005). This research has theorized how reform attempts by regulators and managers typically encounter resistance from professions (Currie, Lockett, Finn, Martin, & Waring, 2012) and documented strategies used by each side to achieve settlements in their favour (e.g., Ackroyd, 1996; Courpasson, 2000). Stated motivations for reform attempts vary widely, including to improve interprofessional collaboration (Bucher, Chreim, Langley, & Reay, 2016); redistribute tasks to lower-status occupations (Reay, Golden-Biddle, & Germann, 2006); and improve service to clients (Kellogg, 2009). But at the core of each of attempt is a desired change to established professional practices (Kellogg, 2009). A key theme in much of this work is knowledge—its nature and relevance to professionals' work as well as who possesses it. Outcomes of struggles over knowledge can profoundly alter a profession's work domain. For example, Waring and Currie (2009) studied how medical professionals reacted to UK hospital managers' efforts to implement knowledge management techniques to enhance organizational learning around patient safety, which threatened physicians' professional autonomy. They found that physicians adopted a range of responses that preserved their autonomy but altered the content of their work—and the requisite knowledge to do it—in ways that blurred the boundaries between professional and managerial ways of organizing work.

A profession uses *expert knowledge*—a system of abstractions guiding practical techniques and presumed to be exclusively held by the profession's credentialed members (Freidson, 1986)—to legitimize its jurisdictional control over a work domain. Research on

professions has highlighted that, to maintain jurisdictional control, a profession must convince different audiences—including government regulators who oversee the profession’s activities, managers of organizations in professionals work, and the profession’s clients—that its expert knowledge can be trusted to inform problem diagnosis, solution prescription, and services rendered in ways that address clients’ needs (Muzio et al., 2020; Eyal, 2019). Thus, a profession and its clients are interdependent. Clients depend on the profession to address their needs, while the profession depends on clients’ trust in its expert knowledge to maintain jurisdictional control over its work domain (Huising, 2021; Lewis & Weigert, 1985).

Despite the centrality of clients to professionals’ work, their role in jurisdictional contestation has remained remarkably overlooked (Anteby et al., 2016: 211). This gap represents an important matter of concern for organizational theorists in light of the ongoing “crisis of expertise” (Eyal, 2019), which involves in many instances a rejection of professionals’ work by clients (Epstein, 2008). Additionally, there is a growing prominence of client engagement in initiatives to address persistent and seemingly intractable problems in professional organizations and systems (Mayo et al., 2021; Boivin et al., 2018; Heaphy, 2017; Fayard et al., 2017). Scholars are increasingly recognizing that profession-client relations merit more attention, and recent research has investigated how professionals engage clients to maintain or expand their jurisdictional control (e.g., Chan & Hedden, in-press; Mukherjee & Thomas, in-press; Huising, 2021). But a focus on clients themselves as jurisdictional contestants remains elusive. We therefore theorize the processes leading clients to participate in jurisdictional contestation, and the specific ways in which they do so.



### Blurred boundaries of the client concept

Before going further, we need to clarify what we mean by ‘client’ and acknowledge the blurred boundaries of this concept. In many work domains, it is often not clear who is, and who is not, a client. Some recipients of professional work have not demanded or consented to it, as for instance when psychiatrists hospitalize and treat nonconsenting patients. ‘Clients’ do not pay professionals directly for their work when there is a third-party payer involved, such as the government or an insurance company (Shapiro, 1987). Also, professions use various terms to identify their clients (clients, consumers, patients, service users, etc.) and sometimes use different terms depending on the work domain (e.g., psychologists may say ‘patients’ in healthcare organizations and ‘clients’ in private practice). Being or not being a client is sometimes a political struggle, as with ‘queers’, ‘survivors,’ ‘fat’, and ‘mad’ folks who seek to de-medicalize their condition and refuse to be considered ‘patients’ (Epstein, 2008). Moreover, the balance of power between professionals and clients varies greatly across work domains and contexts, as Freidson (1989: 429) explains:

If clients are relatively few in number, sophisticated, well-organized and politically or economically powerful, the capacity of members of a profession to select the work they do and determine how they do it is markedly limited: their clients will call many of the shots. On the other hand, if clients are many, unorganized, heterogeneous and individually without significant resources of individual power, professionals are in a better position to call most of the shots.

The clients we theorize about are of the second type, which is typical in health-related work domains (Mayo, Myers, & Sutcliffe, 2021). It is also typical of other professional domains such as education (e.g., Neuman & Guterman, 2017), religion (e.g., Gutierrez, Howard-Grenville & Scully, 2010) and law and order (Cullors & Burke, 2018). This type of client can be difficult to distinguish from members of the public, especially when the latter receive services unpredictably or sporadically; third-party payment and government mandates are involved;

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3 and/or the recipient of professional work do not explicitly consent to it (Freidson, 1989; Shapiro,  
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5 1987).

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8 By acknowledging that a set of criteria to reliably distinguish between clients and non-  
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10 clients appears elusive, we embrace the blurred boundaries of the client concept. Accordingly,  
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12 we consider as a client any person on the receiving end of professional work and over whom  
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14 professional prescriptions apply (cf. Freidson, 1986, Hughes, 1958). This definition emphasizes  
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16 the interdependence of professions and clients insomuch as they need each other to exist.  
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20 **Client participation in jurisdictional contestation**

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22 A profession’s clientele is heterogeneous (Chan & Hedden, in-press) and composed of  
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24 segments with different social locations and embodied experiences (Robnett, 2004). Each person  
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26 has a singular history, nourishes specific aspirations and commitments (Creed et al., 2022;  
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28 Voronov & Weber, 2020), and experiences unique needs, even if their needs are shaped by the  
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30 social groups to which they are attached (Barbalet, 1996). The trust of individual clients in the  
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32 expert knowledge of a profession is shaped by a felt alignment between their needs and how a  
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34 profession interprets and responds to them (Fayard, Stigliani, & Bechky, 2017; Huising, 2015).  
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36 Therefore, when a social movement frames a profession’s expert knowledge as technically  
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38 irrelevant, dangerous, and/or morally corrupt, it risks undermining clients’ trust in its expert  
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40 knowledge and disrupting the profession’s jurisdictional control (Eyal, 2019; Gurri, 2018).  
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45 The disruptive framing efforts of a social movement typically rely on the experiential  
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47 knowledge collectively held by its adherents, which forms a distinct epistemic basis on which to  
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49 legitimize an evaluation of the profession’s work as flawed; and to promote a preferable  
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51 alternative, peer-driven institutional project (cf. Diamond, 2013; Taylor, 2000). Thomasina  
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53 Borkman (1976: 446), a pioneering sociologist of mutual aid, has defined *experiential knowledge*  
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3 as a “concrete, specific, and commonsensical (...) wisdom and know-how gained from personal  
4 participation in a phenomenon.” Her research indicates that a person’s experiential knowledge is  
5 validated by the “conviction that the insights learned from direct participation in a situation are  
6 truth, because the individual has faith in the validity and authority of the knowledge obtained by  
7 being part of a phenomenon” (p. 447). Research on mutual aid groups has shown that the  
8 construction of experiential knowledge is a collective, interpretive process involving peers giving  
9 meaning to their lived experiences through reciprocal sharing of first-person accounts in  
10 dedicated spaces (Jouet, Flora, & Las Vergnas, 2010; Epstein, 2008).

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12 The social movements we have in mind are of this kind. They emerge from marginalized  
13 identity-based communities seeking to emancipate themselves from internalized conceptions of  
14 being flawed and dependencies on others, including professionals, that are taken for granted.  
15 They validate the experiential knowledge of individual clients and help clients to reinterpret their  
16 situation as being caused by societal prejudice and systemic injustice in which a profession’s  
17 expert knowledge is deeply implicated, according to movement adherents (e.g., Gould, 2001;  
18 2009; Schrock, Holden & Reid, 2004; Britt and Heise, 2000). Because this consciousness raising  
19 and mobilization is commonly described as a highly emotional process, we next consider how  
20 emotions lead an individual client to participate in jurisdictional contestation.

### 21 22 **Role of emotions in client participation**

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24 Studies of professions have begun to consider how social emotions shape inter-profession  
25 competition (e.g., Goodrick, Jarvis, & Reay, 2020; Chreim, Langley, Reay, Comeau-Lavallée, &  
26 Huq, 2020). But so far, most research on client-profession relations has not used an emotions  
27 lens (e.g., Evans, 2021; Huising, 2021; Chan & Hedden, in-press; but see Mukherjee & Thomas,  
28 in-press). A recent effort to theorize emotions in jurisdictional contestation by clients was made  
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in Wang et al.’s (2021) study of Chinese patients’ changing attitudes towards physicians. The study finds that a market-oriented reform of government financing for physicians’ work incentivized physicians to boost their income by overprescribing, which undermined patients’ trust in the profession’s expert knowledge. As a result, patients who used to proudly call physicians “angels in white” began shaming them as “wolves in white.” This shift, driven by widespread anger, led to a wave of aggressions by patients against physicians.

These recent findings echo early work by Everett Hughes, a canonical figure for scholars of professions (Chapoulie, 1996; Abbott, 1997), who wrote that there is an “aggressive suspicion which most laymen feel towards professionals, and fanatical anger which burns chronically in some people and which at times becomes popular reaction” (Hughes, 1959: 27). With these colorful remarks, Hughes implied that a profession’s ability to control a work domain depends on the extent to which clients trust the profession’s expert knowledge to appropriately inform the interpretation of, and prescribed response to, their felt needs. Hughes’ remarks also implied that client participation in jurisdictional contestation is an intensely emotional social process.

Emotions shaping client participation in jurisdictional contestation have been most abundantly documented in studies of social movements challenging professions (e.g., Gould, 2009; Britt & Heise, 2000; Bayer, 1987). Anger, for instance, led to the mobilization of activists who challenged the medical and pharmaceutical professions in the early years of the HIV/AIDS pandemic, leading patients to push back against health professionals by setting up their own care and treatment organizations (Maguire, 2002). In this episode, and many others henceforth cited, a highly emotional framing contest arose between a challenger social movement and an incumbent profession to gain the trust of clients in their respective forms of knowledge and to lead them to support their disruptive and defensive institutional projects, respectively.

We build upon emerging literature on clients in the relational perspective on professions (Chan & Hedden, in-press; Huising, 2021; Eyal, 2019) by theorizing how individual clients come to participate in jurisdictional contestation. We do so by mapping distinct configurations of social emotions, experienced by clients exposed to a movement/profession framing contest, to five modes of participation enacted by clients in episodes of jurisdictional contestation. We call these modes *escape*, *opposition*, *accommodation*, *acquiescence*, and *stewardship*.

### **EMOTIONS IN FRAMING CONTESTS: A MODEL OF CLIENT PARTICIPATION IN JURISDICTIONAL CONTESTATION**

Our model draws upon framing theory (Goffman, 1974; Cornelissen & Werner, 2014), according to which frames are “strategic rhetorical devices by which one actor seeks to define a situation for another and influence their thinking and behavior” (Giorgi, 2017: 713). Framing efforts are sustained attempts by actors to promote frames that influence the members of an audience to act in ways favorable to them (Giorgi & Weber, 2015). Framing efforts resonate when the frames promoted by an actor “match or align with the audience’s beliefs, values, aspirations, or ideas” (Giorgi, 2017: 712) in ways that generate intended actions. Framing efforts resonate cognitively when they influence a person’s “thinking,” and emotionally when they influence a person’s “feeling” (Giorgi, 2017: 717). A “framing contest” is a struggle between actors with diverging commitments, each enacting “highly political framing practices to make their frames resonate and to mobilize action in their favor” (Kaplan, 2008: 729).

Building on these concepts, we theorize an episode of jurisdictional contestation as a framing contest in which a social movement acts as challenger by promoting an alternative, peer-driven institutional project that disrupts the jurisdictional control of a profession, which acts as an incumbent by defending the jurisdictional control of its members and established institutional arrangements. We posit that an episode of jurisdictional contestation is initiated when a social

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movement frames the profession’s expert knowledge as flawed and promotes an alternative, peer-driven institutional project guided by the experiential knowledge collectively held by movement adherents. This leads the profession, in turn, to defend its jurisdictional control by reciprocally framing the experiential knowledge guiding the social movement’s alternative institutional project as flawed. Our theory building highlights how the movement and the profession each seek to evoke emotions in ways that strengthen the client’s trust in their respective form of knowledge and gain the client’s attachment to them as a collective actor.

Before proceeding further, we distinguish emotional evocation from emotional resonance to help specify our model’s building blocks and how they fit together. By *emotional evocation*, we refer to collective actors engaging in framing efforts aimed at causing particular emotions in individual members of a target audience so as to lead them to act in specific ways (e.g., Moisander, Hirsto & Fahy, 2016). In contrast, by *emotional resonance* we refer to the emotions experienced by individual members of a target audience as a result of being exposed to the framing efforts of collective actors. Critically, it is these experienced emotions that lead individual members of a target audience to act in specific ways (Giorgi, 2017). Our approach is novel because social studies of emotions tend to focus on either emotional evocation or emotional resonance, but rarely both. Considering evocation together with resonance is important because, by bridging levels of analysis, it enables an explanation of how emotional experience shapes a person’s participation in collective action.

Building on research that theorizes the emancipation of members of marginalized identity movements as an emotional process in which participants move through experiences of fear and anger to convert their shame of individual deviance into a pride of collective affirmation (Britt and Heise, 2000), we focus on a specific constellation of four emotions—pride, anger, shame,

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3 and fear, as well as ambivalence when these emotions overlap in conflicting ways. Further,  
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5 empirical studies of emotional evocation in the framing efforts of challenger movements (cf.  
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7 Bayer, 1987; Whittier, 2017) and incumbent professions (cf. Goodrick et al., 2020; Wright,  
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9 Zammuto, & Liesch, 2017) both frequently mention pride, anger, shame, and fear as the  
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11 emotions that these collective actors seek to evoke in members of their target audiences.  
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14 Likewise, research on the emotional resonance of disruptive (cf. Britt & Heise, 2000; Gould,  
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16 2009) and defensive (cf. Gill & Burrow, 2018; Kish-Gephart, Detert, Treviño, & Edmondson,  
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18 2009) framing efforts in episodes of jurisdictional contestation often note that different forms of  
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20 pride, anger, shame, and fear are experienced by individual members of a target audience.  
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24 The four emotions making up this constellation are “social emotions” because their  
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26 experience shapes the participation of people in social relations and institutional arrangements  
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28 (Barbalet, 1996; Lok, Creed, DeJordy & Voronov, 2017). Fear is felt when someone perceives a  
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30 threat to their well-being combined with a sense of inability to respond adequately to the  
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32 challenge (Gill & Burrow, 2018). Felt shame is described as “a person’s experience of negative  
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34 self-evaluations based on anticipated or actual deprecation by others owing to a failure to meet  
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36 standards of behavior” (Creed et al., 2014: 276). Felt anger is associated with a “moral shock”  
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38 resulting from the perception that a “deserved status” is “denied or withdrawn” (Reger, 2004:  
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40 212). And pride is felt “when one feels positively evaluated by self or others” (Britt & Heise,  
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42 2000: 253).  
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47 Drawing on literatures considered so far, we theorize a framing contest model of how this  
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49 constellation of social emotions shapes the participation of clients of health-related professions in  
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51 episodes of jurisdictional contestation. First, we theorize how, through adversarial framing  
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53 efforts, a social movement acting as challenger and a profession acting as incumbent each seek to  
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3 evoke emotions in particular ways to shape the actions of clients in their favor. Second, we  
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5 explore how the emotional resonance of this framing contest, in the form of specific  
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7 configurations of experienced emotions in individual clients exposed to it, leads them to support,  
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9 to varying degrees, one or both contestants.  
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13 **EMOTIONAL EVOCATION SOUGHT BY FRAMING EFFORTS**  
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15 In this section, we consider how a social movement and a profession involved in an  
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17 episode of jurisdictional contestation engage in adversarial framing efforts that seek to evoke  
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19 emotions in particular ways among clients. Table 1 summarizes our arguments.  
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22 INSERT TABLE 1 ABOUT HERE  
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24 Fligstein and McAdam’s (2012) conflict-oriented theory of fields emphasizes the power  
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26 imbalance between marginalized challengers who seek to disrupt the institutional status quo and  
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28 dominant incumbents who seek to defend it. It suggests that the emphasis on particular emotions  
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30 evoked in the framing efforts of these collective actors may differ depending on their pursuit of a  
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32 project of change or continuity, respectively. Empirical studies tend to corroborate this view,  
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34 indicating that disruptive framing efforts of a challenger tend to emphasize pride of supporting  
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36 its alternative institutional project and anger at the incumbent for breaching its duty (cf. Gamson,  
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38 1992; Gould, 2009), while defensive framing efforts of an incumbent tend to emphasize shame  
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40 of deviating from its prescriptions and fear of relying on the challenger’s alternative project (cf.  
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42 Gill & Burrow, 2018; Creed et al., 2014). These different emphases may, in turn, affect how the  
43  
44 framing efforts of challengers and incumbents resonate in the emotional experience of clients  
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46 exposed to them. We unpack these insights in the next two sections.  
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## Disruptive framing efforts of social movements

A vast, dispersed literature dating from the 1960s (e.g., Cleaver, 1968; Bayer, 1987; Reinecke & Ansari, 2020) documents how social movements have framed the expert knowledge of incumbent actors as technically irrelevant, dangerous and/or morally corrupt, and hence untrustworthy to address adherents' needs. While also valorizing the movement's experiential knowledge as trustworthy, these framing efforts seek to detach targeted audiences from incumbents and gain attachment to the movement (Bayer, 1987; Britt & Heise, 2000; Gould, 2009). This research describes the process by which a person re-evaluates the trustworthiness of different forms of knowledge and their attachment to different collective actors as an intensely emotional one (Whittier, 2001; Taylor, 2000; Hughes, 1959).

More precisely, this body of research documents how the disruptive framing efforts of social movements seek to evoke pride and anger in ways that lead members of their target audiences to work in favor of institutional change (Jasper, 2011; Gamson, 1992). We therefore argue that a social movement involved in jurisdictional contestation engages in disruptive framing efforts to evoke in clients a pride in supporting the alternative, peer-driven institutional project it promotes. It does so by framing the experiential knowledge collectively held by its adherents as relevant, safe, and/or reliable (cf. Borkman, 1999; Epstein, 2008). By seeking to evoke this form of pride in individual clients, the movement attempts to gain the trust of clients in the experiential knowledge of its adherents. A bond of shared trust in the knowledge of experiential peers has been found to provide movement adherents with meaning, validation, and self-worth (Baumeister, Dale, & Muraven, 2000). For example, pride of supporting the alternative project of a trustworthy community of peers is evoked throughout *The womanly art of breastfeeding*, the bestselling guide by La Leche League for women to nurse their infants naturally, without doctor supervision (Wiessinger, West, & Pitman, 2010; Weiner, 1994).

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We also argue that a movement involved in jurisdictional contestation engages in framing efforts to evoke anger in clients at the profession for breaching its duty towards them (cf. Smith & Freyd, 2014). It does so by framing deviance from the profession’s prescriptions as a righteous imperative (cf. Whittier, 2017; Lindebaum & Gabriel, 2016). By seeking to evoke this form of anger in individual clients in relation to the profession, movement adherents attempt to gain the attachment of clients to the movement as a collective actor (cf. Hochschild, 1975; Taylor, 2000). Attachment to the movement has been linked to a “we” versus “they” moral dichotomy (Gamson, 1992: 84) that is constructed and reinforced by the movement’s framing efforts. For example, adherents of the LGBT movement have sought to evoke the anger of their peers and direct it at the profession of psychiatry for pathologizing what they consider to be their authentic and unproblematic selves (Bayer, 1987; Schrock et al., 2004).

Research on social movements also indicates that their framing efforts seek to evoke shame and fear in ways that shape how members of their target audiences participate in the contestation of established arrangements (Gould, 2009; Bayer, 1987). We argue that a movement involved in jurisdictional contestation engages in disruptive framing efforts to evoke in clients a shame of complying with the profession’s prescriptions, framing compliance as a moral flaw and/or a self-degradation (cf. Schrock et al., 2004). By seeking to evoke this form of shame in individual clients in relation to the profession, movement adherents attempt to cause the detachment of clients from the profession as a collective actor. For example, in the first and last pieces of his collection of essays *Melancholia and moralism*, queer activist Douglas Crimp (2004) shames the journalist Andrew Sullivan, who lives with HIV/AIDS like him, as well as Sullivan’s supporters for being too supportive of medical professionals, the pharmaceutical industry and the pharmacy profession.

Drawing on these studies we also argue that a movement involved in jurisdictional contestation engages in disruptive framing efforts to evoke in clients a fear of relying on established arrangements upheld by the profession. It does so by framing the profession's expert knowledge as irrelevant, dangerous, and/or unreliable (cf. Taylor, 2000). By seeking to evoke this form of fear in individual clients in relation to the profession, movement adherents attempt to undermine the trust of clients in the expert knowledge on which rests the profession's jurisdictional control over established arrangements. For example, the framing efforts of mad activists, as evidenced by abundant texts that they have authored (e.g., LeFrançois, Menzies, & Reaume, 2013; Shimrat, 1997; Chamberlin, 1977), clearly seek to evoke fear in patients of being diagnosed, confined, and treated against their will by psychiatrists, thus highlighting this movement's efforts to undermine the trust of patients in the expert knowledge of the profession.

In sum, we build on social studies of emotions to argue that social movements involved in jurisdictional contestation engage in disruptive framing efforts to evoke *pride* and *anger* in clients in ways that strengthen the trust of clients in the movement's experiential knowledge and gain the attachment of clients to the movement, respectively. Additionally, we argue that these movements also engage, although to a lesser extent, in disruptive framing efforts to evoke *shame* and *fear* in ways that cause the detachment of clients from the profession as a collective actor and undermine the trust of clients in the profession's expert knowledge, respectively.

### **Defensive framing efforts of professions**

Empirical studies indicate that, when established arrangements are contested, incumbent actors such as professions engage in defensive framing efforts to evoke fear and shame in forms that lead members of their target audiences to work in support of continuity (Gill & Burrow, 2018; Creed et al., 2014). Building on this insight, we argue that a profession whose

jurisdictional control is contested by a social movement engages in framing efforts to evoke in clients a fear of relying on the alternative project promoted by the movement. It does so by framing the movement’s experiential knowledge as irrelevant, dangerous, and/or unreliable (cf. Chesler, 1987; Edelman, 1974). By seeking to evoke this form of fear in individual clients in relation to the movement, profession members attempt to undermine the trust of individual clients in the experiential knowledge on which rests the movement’s alternative project (cf. DeCelles, Sonenshein, & King, 2019). For example, a study of childbirth experiences suggests that obstetrics professionals seek to evoke fear in pregnant women of complications related to giving birth at home accompanied by peers, thereby attempting to undermine the trust of these women in the home birth movement (Stoll, Fairbrother, & Thordarson, 2018).

Similarly, drawing on these studies we suggest that a profession faced with jurisdictional contestation engages in defensive framing efforts to evoke in clients a shame of deviating from its prescriptions. The profession does so by framing deviance from its prescriptions as a moral flaw and/or a self-degradation (cf. Creed et al., 2014; DeJordy & Barrett, 2014). By seeking to evoke this form of shame in individual clients in relation to the movement, profession members attempt to cause their detachment from the movement as a collective actor. For example, medical professionals seek to shame patients in whom they diagnose obesity into compliance with the profession’s eating, exercising, and treatment prescriptions (AAMC, 2022), attempting to cause these patients’ detachment from the fat acceptance movement (Cooper, 2016).

Studies also indicate that incumbent actors facing contestation seek to evoke anger and pride in forms that lead members of their target audience to work in support of continuity (Wright et al., 2017; Massa, Helms, Voronov, & Wang, 2017). Thus, we argue that a profession facing jurisdictional contestation by a social movement engages in framing efforts to evoke

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3 anger in clients at the movement for jeopardizing a proper response to their needs. It does so by  
4 framing compliance with the profession's prescriptions as a righteous imperative (Wright et al.,  
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6 2017; Haidt, 2003). By seeking to evoke this form of anger in individual clients in relation to the  
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8 movement, profession members attempt to gain the attachment of clients to the profession as a  
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10 collective actor. For example, insider researcher Deborah Gould (2009) recounts how the  
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12 medical profession and pharmaceutical industry sought to evoke the anger of people living with  
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14 HIV/AIDS and direct it at ACT/UP activists whose disruptive protest tactics, the medical  
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16 profession claimed, compromised the development and distribution of treatments that people  
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18 living with HIV/AIDS urgently needed. In doing so, the profession was attempting to gain the  
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20 attachment of individual clients in it as a responsible collective actor trying to uphold established  
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22 arrangements which, the profession argued, benefited people living with HIV/AIDS.  
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28 We also argue that a profession faced with jurisdictional contestation engages in  
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30 defensive framing efforts to evoke in clients a pride of supporting established arrangements. It  
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32 does so by framing the profession's expert knowledge as relevant, safe, and/or reliable (cf.  
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34 Lawrence, 2017). By seeking to evoke this form of pride in individual clients in relation to the  
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36 profession, professionals attempt to strengthen the trust of clients in the profession's expert  
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38 knowledge. This bond of trust in expert knowledge provides supportive audience members with  
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40 a reassuring sense of being cared for by a dutiful collective of professionals (Evetts, 2006; Lewis  
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42 & Weigert, 1985). For example, during the Covid-19 pandemic, healthcare professionals  
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44 presented themselves in the media as selfless "saviors" by telling their inspiring stories of  
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46 dutifully working "at the forefront, keeping themselves and their families at risk just to treat this  
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48 disease and save lives" (Kumar, Bharti, Prakash, & Kumar, 2022). Such framing efforts seek to  
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3 evoke the pride of citizens in supporting the healthcare professions, as well as to cultivate the  
4 public’s trust in their expert knowledge amid the global pandemic.  
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8 In sum, we build on social studies of emotions to argue that professions facing  
9 jurisdictional contestation engage in defensive framing efforts to evoke *fear* and *shame* in ways  
10 that undermine the trust of clients in the experiential knowledge of a challenger movement and  
11 cause the detachment of clients from the movement, respectively. Additionally, we argue that  
12 professions in such situations also engage, although to a lesser extent, in defensive framing  
13 efforts to evoke *anger* and *pride* in ways that gain the attachment of clients to the profession and  
14 strengthen their trust in its expert knowledge, respectively.  
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24 **EMOTIONAL RESONANCE AND MODES OF CLIENT PARTICIPATION**  
25 **IN JURISDICTIONAL CONTESTATION**  
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28 In this section, we theorize (1) how the social movement and profession’s framing  
29 efforts, when combined as part of a framing contest, can be more or less successful at evoking  
30 specific emotions in individual clients; and (2) how the emotional resonance of this framing  
31 contest in clients exposed to it leads them to enact one of five modes of participation in  
32 jurisdictional contestation. Table 2 summarizes our arguments.  
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40 INSERT TABLE 2 ABOUT HERE  
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43 Before describing and illustrating each mode, we emphasize the role of felt ambivalence  
44 in shaping client participation in jurisdictional contestation. Research on social emotions  
45 highlights the prominence of ambivalence in episodes of contestation (Creed, DeJordy, & Lok,  
46 2010; Meyerson & Scully, 1995). Ashforth, Rogers, Pratt & Pradies (2014: 1454) defined felt  
47 ambivalence as “simultaneously positive and negative (...) actor’s alignment or position with  
48 regard to [an] object, where a positive orientation means attraction or a pull toward it and a  
49 negative orientation means repulsion or a push away from it.” Meyerson and Scully (1995: 588)  
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3 wrote that ambivalence “stems from the Latin *ambo* (both) and *valere* (to be strong)” and implies  
4 the “expression of both sides of a dualism.” Full contestation of, and support for, a profession’s  
5 jurisdictional control are ideal types that represent opposite ends of a continuum. Empirically, we  
6 argue that it is common for clients exposed to an episode of jurisdictional contestation to  
7 experience various degrees and forms of ambivalence—i.e., most clients neither fully embrace or  
8 repudiate the movement’s aspirations nor the profession’s jurisdictional control.  
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11 For instance, a client may feel angry at a profession for its perceived breach of duty and  
12 therefore inclined to support contestation, yet also feel afraid of relying solely on a movement’s  
13 alternative institutional project to address their needs, which would temper their support  
14 (DeCelles et al., 2019; Kish-Gephart et al., 2009). In other instances, a client may feel proud of  
15 supporting the alternative project promoted by the movement, but also ashamed of deviating  
16 from the profession’s prescriptions (Gould, 2001; Britt & Heise, 2000). Thus, we argue that felt  
17 ambivalence profoundly shapes how a client participates in jurisdictional contestation.  
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20 Building upon this argument, we theorize five distinct modes of client participation in  
21 jurisdictional contestation. In developing our modes, we sought a continuum that not only  
22 connected extreme and opposite client reactions to the framing contest but was also informed by  
23 the insights, detailed above, that challengers emphasize pride and anger while incumbents  
24 emphasize fear and shame, even if both sides seek to evoke all four emotions to some degree but  
25 in different ways.<sup>1</sup> So, we began with the intent of identifying one mode for each situation in  
26 which one of these emphasized emotions, i.e., the types of pride and anger sought by the  
27 challenger movement and the types of fear and shame sought by the incumbent profession, had  
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<sup>1</sup> We are grateful to our reviewers for encouraging the symmetric treatment of both sides of the framing contest as concerns emotions sought for evocation.

strong resonance and for which the action taken by a client could be causally linked to it. Next, we anchored the two ends of our continuum on full success for each side in the framing contest in terms of achieving resonance of the emotions they sought to evoke. These steps led us to “escape” and “stewardship”. We then incorporated ambivalence by considering situations where one side had minimal but non-zero success across the four emotions sought for evocation in terms of achieving resonance, which led us to “opposition” and “acquiescence”. This step led us to realize that we also needed to consider situations where both sides were equally successful across the four emotions sought for evocation and where two strongly resonating emotions pulled in divergent directions, which led us to two distinct types of “accommodation”.

At each step in which we settled on a mode, we complemented our theoretical moves with considerations of the empirical support that we could find in research describing social movements disrupting the jurisdictional control of health-related professions. By interrogating this research, we noted that, indeed, more and less successful framing efforts by a social movement (and, conversely, a profession), defined in terms of achieving strong, some or no resonance of the emotions it sought to evoke, empirically mapped to different modes of client participation and different constellations of emotions experienced by clients. Although other configurations of emotional resonance and possible actions taken by clients are imaginable, this step of triangulating our theoretical inferences with empirical evidence gave us confidence in the ones which we included in our model—of which Figure 1 provides an overview.

INSERT FIGURE 1 ABOUT HERE

*Escape* and *stewardship* represent the two least ambivalent modes of client participation, with each at opposite ends of the continuum of supporting movement or profession, respectively. The *opposition*, *accommodation*, and *acquiescence* modes are shaped by different forms of felt



ambivalence where the framing efforts of both movement and profession resonate to some degree in clients. We illustrate each mode with examples from health-related work domains.

## Escape

The mode of client participation in jurisdictional contestation we call ‘escape’ is shaped by the exclusive emotional resonance of a movement’s disruptive framing efforts. By escape, we mean withdrawing from established arrangements to pursue a peer-driven, alternative project. We argue that clients enact the escape mode of participation because they feel very proud of supporting the movement’s alternative, peer-driven institutional project, as well as somewhat afraid of relying on the profession’s established arrangements, ashamed of complying with the profession’s prescriptions, and angry at the profession for breaching its duty. Clients who experience this configuration of emotional resonance come to see the movement’s experiential knowledge as highly trustworthy. They become strongly attached to the movement as a collective actor. This configuration of emotional resonance is illustrated in Figure 2.

INSERT FIGURE 2 ABOUT HERE

Exemplifying this mode of participation are adherents to what has been variously known since the 1970s as the psychiatric survivors or mad movement. Judi Chamberlin’s *On our own* (1977) is a profoundly emotional first-person account of having “survived” what she describes as oppressive psychiatric treatments. It is also a forceful manifesto advocating for the “liberation” of psychiatric patients through their mobilization for an alternative institutional project of mutual aid among experiential peers. In the conclusion of this text, which to this day remains a foundational reference for movement adherents, Chamberlin (1977: 239) wrote:

We must support the establishment of neighbourhood crisis centers, drop-in centers, community residences, and group homes (...). And we must welcome back into our communities even those people so damaged by years of psychiatric incarceration that they may always remain a bit different, a little odd.

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Through multiple passages like this one, Chamberlin communicates her own pride and that of her peers in supporting the alternative project promoted by the psychiatric survivors’ movement and her trust in the experiential knowledge of her fellow movement adherents: “People who have been patients know from their own experience that warmth and support (...) were helpful and that being thought of and treated as incompetent were not. Even when a person is experiencing distress he or she still can be helpful to others,” she writes (p. 6).

Recent research by mad activists has noted that this pride of supporting the movement’s alternative project is cultivated among movement adherents through “[p]ersonal narratives of surviving, outwitting, or avoiding psychiatry [which] appear regularly in movement publications and provide an important segment of programming at annual conferences” (Morrison, 2005: 101). Such “heroic survivor narratives” strengthen the trust of adherents in their experiential knowledge. In the introduction of her manifesto (1977: xv), Chamberlin also wrote:

Many ex-patients are angry, and our anger stems from the neglect, indifference, dehumanization, and outright brutality we have seen and experienced at the hands of the mental health system. Our distrust of professions is not irrational hostility, but is the direct result of their treatment of us in the past. We have been belittled, ignored, lied to. We have no reason to trust professionals, and many reasons to fear them.

In this quote, Chamberlin describes the fear felt among movement adherents at the idea of relying on the established arrangements upheld by the psychiatric profession and their distrust of its expert knowledge which they see as irrelevant, dangerous, and unreliable. She also states the anger felt by movement adherents towards the profession for breaching its duty—an anger at ‘them’ that attaches adherents to the ‘we’ of the movement and leads them to consider deviance from the profession’s prescriptions as a righteous imperative. Research on the history of the mad movement describes how the shame of complying with the profession’s prescriptions led mad activists to become detached from the psychiatric profession and to see compliance with its

prescription as morally flawed and self-degrading, creating tensions between the movement's radical and reformist segments (Campbell, 2011; Morrison, 2005).

Similar emotional dynamics have been described among the feminist founders of a peer-driven crisis center for rape survivors (Zilber, 2002), as well as in mutual aid groups for women who practice breastfeeding and proximal mothering (Weiner, 1994), transgendered people (Schrock et al., 2004), and stutterers (Borkman, 1999). Fueled by pride in their newfound community but also specific forms of anger, shame and fear, adherents to these movements seek to withdraw from a profession's jurisdiction to pursue alternative, peer-driven arrangements.

## **Opposition**

'Opposition' refers to a mode of client participation that arises when the framing efforts of both movement and profession resonate emotionally in a client to some degree, but where the movement's disruptive framing efforts dominate. By opposition, we mean that clients seek to radically transform the established arrangements upheld by the profession in order to redress the immorality of these arrangements. Opposition is pursued, we argue, by clients who feel very angry at the profession for breaching its duty as well as somewhat ashamed of complying with the profession's prescriptions and proud of supporting the movement's alternative project, yet nonetheless somewhat afraid of relying on the movement's alternative project to address their needs. As a result of this form of felt ambivalence (very angry at the profession but somewhat fearful of the movement's project), clients become attached to the movement and consider deviance from the profession's prescriptions as a righteous imperative, while also remaining engaged with the established arrangements upheld by the profession because they do not fully trust the movement's experiential knowledge to address their needs. This configuration of emotional resonance is illustrated in Figure 3.

INSERT FIGURE 3 ABOUT HERE

For example, birth justice activists angrily denounce the obstetrics profession as prejudiced because “[p]regnancy experiences and outcomes can be dramatically different for (...) women of color, immigrants, LGBTQ people, disabled women, and those with low incomes, among others” (ANSIRH, 2022). Peer-led groups where women share with each other their first-person accounts of obstetrical violence (cf. Obstetric Justice Project, n.d.) nurture a woman’s attachment to the movement as a collective actor by creating a moral dichotomy (Gamson, 1992) between the movement and the obstetrics profession. Movement adherents seek to transform practices of the obstetrics profession that, in their view, mistreat and harass birthing people and their babies (cf. Fatima, 2020). They consider denouncing this systemic problem and advocating for radical change in obstetrical practices as righteous imperatives.

The anger of adherents combines with a felt pride in supporting the movement’s righteous cause “to safeguard the rights of birthing people to make decisions around their pregnancies, including seeking abortions, and to have children in safe and supportive environments” (ANSIRH, 2022). Such a pride strengthens the trust of adherents in the movement’s experiential knowledge. Adherents may also feel ashamed if they perceive themselves to be insufficiently supportive, in the eyes of their peers, of a movement that defends their collective needs against a profession breaching its duty to clients.

However, birth justice activists remain engaged with the obstetrics profession despite their misgivings about its expert knowledge, which is why they feel compelled to denounce the profession *and* work to transform its practices at the same time. This stance suggests a fear of relying solely on the alternative project of giving birth at home with peers (cf. Zielinski, Ackerson, & Low, 2015). Feeling some fear in relation to the movement’s project reflects

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3 adherents' incomplete trust in their shared experiential knowledge and related doubts about the  
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5 movement's ability to fully address their needs. Consequently, and despite strong anger at the  
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7 obstetrics profession, birth justice activists do not seek to desert established arrangements but,  
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9 instead, seek to radically redress their immorality and make them more relevant, safer, and more  
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11 reliable for them and their peers.  
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### 14 15 **Accommodation** 16

17         The mode of client participation we refer to as 'accommodation' is enacted by clients in  
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19 whom the contestants' adversarial framing efforts have comparable degrees of emotional  
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21 resonance. Although different forms of ambivalence may lead to accommodation, in each, clients  
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23 experience some degree of anger and pride as evoked by the movement's framing efforts, yet  
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25 also some degree of fear and shame as evoked by the profession's framing efforts.  
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28         We argue that accommodation is enacted by clients who feel very angry at the profession  
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30 for breaching its duty, yet also equally fearful of relying on the movement's alternative project to  
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32 address their needs. By accommodation, we mean that clients seek to incrementally improve  
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34 established arrangements. Clients experiencing this configuration of emotional resonance are  
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36 attached to the movement as a collective actor but mistrust its experiential knowledge to address  
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38 their needs. We call this form of ambivalence towards the movement 'attachment yet mistrust'. It  
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40 leads clients to see deviance from the profession's prescriptions as a righteous imperative, yet  
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42 nonetheless remain engaged with established arrangements and work to incrementally improve  
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44 them. Enacting accommodation in this way allows clients to reconcile contradictory inclinations  
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46 towards the movement. This configuration of emotional resonance is illustrated in Figure 4a.  
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Exemplifying this form of ambivalence towards the movement, which leads clients to enact accommodation, are adherents to the patient partnership movement, which gained a foothold across healthcare systems in recent decades. Mobilized by a latent anger at healthcare professions and their practices, which they see as paternalistic and exclusionary, members of patient groups including hemophiliacs, people living with HIV/AIDS, cancer survivors, mental health advocates, and others, are demanding to be included in the design, governance, evaluation of, and research about health care (Maurer, Dardess, & Rouse, 2019; Jouet et al., 2010).

By enacting accommodation, adherents to the patient partnership movement seek to work from inside established arrangements, collaborating with members of incumbent professions to incrementally improve their practices. This stance, more ‘reformist’ than ‘revolutionary’, suggests that, despite their anger at healthcare professions, patient partners nonetheless maintain some trust in professions’ expert knowledge. Their fear of relying solely on peer-driven, alternative arrangements to address their needs leads them to seek collaboration with healthcare professions and advocate for the valorization and inclusion of their experiential knowledge of illness and care as a way to address the shortcomings of established healthcare arrangements (Boivin, Dumez, Fancott, L'Espérance, 2018; Epstein, 2007).

We argue that accommodation is also pursued by clients experiencing another form of ambivalence. They are clients who feel very proud of supporting the movement’s alternative project, yet equally ashamed of deviating from the profession’s prescriptions. Clients experiencing this emotional resonance trust the movement’s experiential knowledge to address their needs but remain detached from it as a collective actor because they see deviance from the profession’s prescriptions as morally flawed and/or self-degrading. We call this form of ambivalence towards the movement ‘trust yet detachment’. These clients see the movement’s

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2  
3 experiential knowledge as more relevant, safer, and more reliable than the profession's expert  
4 knowledge, yet they remain engaged with the profession's established arrangements and work  
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6 towards incrementally improving them from inside because they do not want to be seen as  
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8 deviant. Enacting accommodation allows them to reconcile these contradictory inclinations  
9  
10 towards the movement. This configuration of emotional resonance is illustrated in Figure 4b.  
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15 INSERT FIGURE 4b ABOUT HERE  
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17 Illustrating the 'trust yet detachment' form of ambivalence towards the movement, which  
18  
19 leads clients to enact the accommodation mode of participation in jurisdictional contestation, are  
20  
21 HIV/AIDS activists. Gould (2009: 24) describes the ambivalence experienced in their struggle  
22  
23 with the medical profession as a mix of "contradictory feeling states, including shame about  
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25 homosexuality along with gay pride, as well as a desire for social acceptance along with  
26  
27 repulsion from a society that oppresses sexual minorities." She notes how the ambivalence of  
28  
29 activists, who feel proud of the movement's change project but ashamed of being seen to support  
30  
31 it and labeled as deviant, leads them to work for incremental change while trying not to 'rock the  
32  
33 boat' and risk marginalization from mainstream arrangements (Gould, 2001). In different ways,  
34  
35 both the 'trust yet detachment' and 'attachment yet mistrust' forms of ambivalence towards the  
36  
37 movement involve in individual clients a desire to challenge prevailing arrangements that is  
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39 tempered by a concern for keeping their place within them.  
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#### 44 45 **Acquiescence**

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47 The mode of client participation we label as 'acquiescence' arises when a client  
48  
49 experiences the emotional resonance of both contestants' framing efforts, but the profession's  
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51 defensive framing efforts dominate. By acquiescence, we refer to clients passively accepting the  
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53 profession's established arrangements despite latent grievances. We argue that clients enact  
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acquiescence when they feel very ashamed of deviating from the profession’s prescriptions as well as somewhat afraid of relying on the movement’s alternative project and angry at the movement for jeopardizing a proper response to their needs, yet nonetheless also somewhat proud of the movement’s change project. Clients feeling ambivalent in this way remain attached to the profession and consider deviance from its prescriptions as wrong, which leads them to passively accept established arrangements despite their trust in the movement’s experiential knowledge. This configuration of emotional resonance is illustrated in Figure 5.

INSERT FIGURE 5 ABOUT HERE

Exemplifying this mode of participation are citizens who complied with and refrained from openly criticizing public health mandates to prevent the propagation of Covid-19 despite privately disagreeing with some aspects of them, suggesting that their desire to avoid being targets of shaming led them to passively accept the arrangements upheld by public health professionals. A report for the Guardian published in the middle of the pandemic observed that on social media, “[t]housands of people [were] blaming, naming, and shaming others for their improper pandemic practices” (Tait, 2020). This created a climate of intense peer pressure that led many citizens to comply with public health mandates in order to avoid being seen as deviant, and to feel somewhat angry at movements that challenged the public health profession, despite silently agreeing with aspects of the challenger movement’s critique and feeling somewhat proud of the movement’s alternative project (cf. Graso, Chen, & Reynolds, 2021). Clients experiencing the framing contest in this way remain detached from the challenger movement, although they partially trust the experiential knowledge of movement adherents.

A variety of studies have documented the mechanisms that lead most people under certain conditions to downplay misgivings and grievances and to passively accept established



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2  
3 arrangements upheld by institutional authorities, thereby contributing to their maintenance.  
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5 These mechanisms include peer pressure, rationalizations, and denial (Delmestri & Goodrick,  
6  
7 2016; Zerubavel, 2006; Ashforth & Anand, 2003), as well as uncritical acceptance of  
8  
9 institutional roles (Clegg, Courpasson, & Phillips, 2006; Goffman, 1983; Marcuse, 1965) and  
10  
11 internalized norms of obedience to seemingly or patently unjust authorities (Milgram, 1971;  
12  
13 Arendt, 1963). The dynamics of clients' acquiescence theorized here complement extant  
14  
15 understandings of passive acceptance by institutional constituents of dissatisfactory  
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17 arrangements, a form of participation that seems ubiquitous in many contexts.  
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## 22 **Stewardship**

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24 The mode of client participation in jurisdictional contestation we call 'stewardship' is  
25  
26 enacted when a client experiences exclusively the emotional resonance of a profession's  
27  
28 defensive framing efforts. By stewardship, we mean clients seeking to actively reinforce the  
29  
30 profession's established arrangements. We argue that clients enact stewardship when they feel  
31  
32 very afraid of relying on the movement's alternative project and also somewhat ashamed of  
33  
34 deviating from the profession's prescriptions, angry at the movement for jeopardizing a proper  
35  
36 response to their needs, and proud of supporting established arrangements. Clients experiencing  
37  
38 this configuration of emotional resonance consider the profession's expert knowledge to be  
39  
40 highly trustworthy and remain fully attached to the profession as a collective actor. This  
41  
42 configuration of emotional resonance is illustrated in Figure 6.  
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47 INSERT FIGURE 6 ABOUT HERE

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49 Exemplifying stewardship are those psychiatric patients who, in response to disruptive  
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51 efforts like those of Judi Chamberlin (1977; see 'Escape' above), advocate for *reinforcing* the  
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53 legal authority of psychiatrists to hospitalize and administer treatments to nonconsenting  
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3 patients. When interviewed, a well-known mental health advocate who identifies as  
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5 ‘schizophrenic’ argued that *greater* coercive authority for psychiatrists would allow them to  
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7 protect more effectively not only psychotic people but also citizens more generally when these  
8  
9 people behave dangerously (Vigneault, 2016). This advocate’s position conveys a strong fear felt  
10  
11 at the prospect of relying on the alternative project promoted by the psychiatric survivors’  
12  
13 movement, as well as mistrust of their experiential knowledge. It also suggests feelings of shame  
14  
15 at the idea of deviating from the profession’s prescriptions and pride in supporting established  
16  
17 arrangements, as well as anger towards disruptors of the psychiatric profession’s jurisdictional  
18  
19 control over its work domain. This advocate’s stewardship of the psychiatric profession’s  
20  
21 jurisdictional control represents an opposite reaction to the project laid out in Chamberlin’s  
22  
23 (1977) manifesto cited earlier and followed by clients who enact the escape mode.  
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28 Different, but also representative of the stewardship mode of participation, are celebrities  
29  
30 representing the Bell Let’s Talk campaign across Canada, who testify to the sense of safety  
31  
32 gained from accepting their mental disorder and seeking appropriate professional treatment to  
33  
34 help them cope with it (Rubin, 2017). These high-profile clients of psychiatry are displaying  
35  
36 their trust in the profession’s expert knowledge (Dhami, 2020). Their personal narratives in  
37  
38 strong support of psychiatry’s jurisdictional control and the safety it provides simultaneously  
39  
40 convey strong fear felt at the idea of relying on an alternative project of mutual aid among peers  
41  
42 (Vido, 2019). In addition to the pride these celebrities take in publicly supporting established  
43  
44 arrangements, their campaign may also reflect some shame at the idea of deviating from the  
45  
46 profession’s prescriptions and anger at movements that disrupt what they see as the psychiatry’s  
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48 proper response to the needs of clients. This configuration of emotional resonance reinforces  
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their trust, as clients, in the profession's expert knowledge and strengthens their attachment to the profession as collective actor.

## DISCUSSION

In this article, we have developed a model of how emotions shape the participation of professions' clients in episodes of jurisdictional contestation. Our theorizing contributes to the literature on professions by conceptualizing client participation in jurisdictional contestation across analytical levels; considering the role of a constellation of four intertwined social emotions in this process; and introducing a typology of five modes of client participation in jurisdictional contestation. We developed our model by drawing on empirical examples from health-related professions and, in this section, we also discuss its generalizability to work domains other than health-related ones and to other stakeholders.

### Clients as jurisdictional contestants

Organizational studies of professions have mainly considered episodes of jurisdictional contestation initiated by regulators and managers (e.g., Ackroyd, 1996; Waring & Currie, 2009). Our paper theorizes efforts from social movements to bring individual clients into collective action to alter the boundaries and content of a profession's work. The model expands relational understandings of jurisdictional control as grounded in how a profession relates to its clientele (Chan & Hedden, in-press; Huising, 2021; Evans, 2021; Eyal, 2019). By foregrounding how jurisdictional control depends on clients' trust in a profession's expert knowledge and attachment to it as a collective actor, we present continuity in jurisdictional control as an ongoing achievement that is imperiled by contestation from a client-led social movement, thus shedding new light on existing literature focused on regulators and managers.

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Notable similarities and differences become apparent when episodes of contestation by regulators and managers are compared with those by client-led social movements. On one hand, they are similar because, in both cases, the jurisdictional control of a profession is disrupted by a collective actor and, as a result, arrangements in a work domain may be altered. Also, in both cases the change project sought by the collective actor initiating the episode is likely to be resisted by the contested profession (Langley et al., 2019). On the other hand, they are different because contestation by regulators and managers produces a struggle between high-status actors wielding distinct forms of formal authority, whereas contestation by a social movement produces a struggle between a high-status actor with formal authority (the profession) and a low-status one without it (the movement). That is, contestation from regulators and managers gives rise to a struggle between powerful incumbents over the boundaries of recognized jurisdictions, while contestation from a social movement gives rise to a struggle between an incumbent profession and a marginalized challenger without any recognized claim to jurisdiction (cf. Fligstein and McAdam, 2012). This asymmetry shifts the dynamics of the struggle. Unlike regulators and managers who seek support from profession members to implement reform (e.g., Kellogg, 2009; Bucher et al., 2016), the social movements we theorize seek to enlist clients in collective action to alter the boundaries and content of a profession’ work (e.g., Gould, 2009; Bayer, 1987).

Our focus on clients adds insights to the literature by foregrounding a basis for jurisdictional contestation that is quite different from the formal regulatory and managerial authority of actors most studied so far (e.g., Courpasson, 2000; Suddaby et al., 2007). Social movements rely on the experiential knowledge of their adherents to gain clients’ support for their alternative projects. This distinctly epistemic basis for contestation is important because it creates unique complications for a profession seeking to defend its jurisdiction by invoking the

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2  
3 authority of its expert knowledge (Eyal, 2019). The means of construction, transmission, and  
4  
5 validation of experiential knowledge are different from those of expert knowledge. They involve  
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7 mutual sharing of first-person accounts within communities of experiential peers, with much of  
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9 this narrative work taking place within dedicated spaces where professionals are not invited  
10  
11 (Borkman, 1999; Epstein, 2008). Within these peer-only spaces, the subjectivity and unicity of  
12  
13 individual experiences are valorized and bind movement adherents to each other, while the  
14  
15 objectivity and generalizability claimed by holders of expert knowledge are problematized and  
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17 often dismissed (Jouet et al., 2010; Taylor, 2000). Social movements are formidable  
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19 jurisdictional contestants because they validate the experiential knowledge of clients and  
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21 encourage them to voice their aspirations for change.  
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26         Regulators and managers have sought to systematize expert knowledge and ease its  
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28 sharing across professional boundaries, typically for reasons of efficiency and organizational  
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30 flexibility (Waring & Currie, 2009; Bucher et al., 2016). Studies of jurisdictional contestation by  
31  
32 regulators and managers present their knowledge as complementary to professionals' knowledge  
33  
34 rather than substitute for it (Scott, Ruef, Mendel, & Caronna, 2000). In contrast, social  
35  
36 movements promote the use of adherents' experiential knowledge to address clients' needs as a  
37  
38 legitimate substitute for expert knowledge in at least some important aspects of the profession's  
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40 work domain. This unique basis for jurisdictional contestation has important implications for  
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42 how episodes of contestation play out, including the role of emotions in them.  
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### 47 **Emotions in jurisdictional contestation**

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49         The experiential epistemic basis for client contestation of a profession's jurisdiction  
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51 results in a more prominent role for emotions as compared to regulators and managers. This  
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53 difference likely arises because the construction, validation, and transmission of expert  
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3 knowledge is guided by ideals of distance and objectivity which invalidate emotional experience  
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5 as a way of knowing and repress its display (Collinson, 2003; Fournier, 1999). In contrast,  
6  
7 adherents to social movements valorize emotional experience as a form of knowledge about self  
8  
9 and society (Borkman, 1999). Movement adherents encourage their peers to display pride  
10  
11 openly, as a mechanism for bonding and affirmation (Taylor; 2000; Schrock et al., 2004); while  
12  
13 engaging in collective outbursts of anger is a well-known social movement tactic for getting the  
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15 attention of incumbents whose practices are targeted for change such as governments,  
16  
17 corporations, and professions (Bayer, 1987; Gould, 2009). Thus, our model highlights the central  
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19 role of social emotions when clients contest a profession’s jurisdiction.  
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24       Most organizational research on jurisdictional contestation of professions, in contrast, has  
25  
26 not focused on social emotions. Studies focusing on contestation by regulators and managers  
27  
28 have mostly overlooked the emotions of contestants, describing struggles between collective  
29  
30 actors pursuing divergent institutional projects as driven by power or efficiency considerations  
31  
32 (e.g., Scott et al., 2000; Brint, 1994). A few recent studies of jurisdictional contestation exploring  
33  
34 the role of clients have begun to integrate emotions. In their study of profession stigmatization,  
35  
36 Wang et al. (2021) found that individual clients angry at the medical profession sought to alter  
37  
38 the profession’s prescribing practices by evoking shame in medical professionals. However, the  
39  
40 phenomenon described by the authors is different to the one we theorize; no social movement is  
41  
42 mentioned and, instead, news media are credited with amplifying the conflict although, and  
43  
44 consistent with our model, felt anger led clients to oppose the profession. Mukherjee & Thomas  
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46 (in-press) find that palliative care professionals manage ‘feeling rules’ that define what forms of  
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48 fear, hope and guilt constitute an ‘appropriate’ emotional response from clients to the  
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50 profession’s work. This study suggests that in their everyday work professions seek to  
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3 evoke—and are generally successful at achieving resonance of—specific forms of emotions  
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5 which *preclude* jurisdictional contestation by clients.  
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8         So, despite some recent attention to emotions and professions, an integrative treatment of  
9  
10 social emotions has not informed jurisdictional contestation research. Our theorizing of how  
11  
12 social emotions shape client participation in the contestation of a profession's jurisdictional  
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14 control is therefore novel and advances research in multiple ways. First, it brings together  
15  
16 insights scattered across a vast, heterogeneous literature, including studies of emotions in social  
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18 movements and their struggles against opponents (e.g., Gould, 2001; 2009; Schrock et al., 2004;  
19  
20 Britt and Heise, 2000). This voluminous work, however, does not consider the implications of its  
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22 findings for professions and the work domains they control, as we have done in this paper.  
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26         Second, our model adds a nuanced understanding of social emotions to research on  
27  
28 jurisdictional contestation by considering a constellation of four intertwined social emotions;  
29  
30 theorizing both sides of a framing contest to gain clients' support; and distinguishing between,  
31  
32 but also bringing together, emotional evocation and resonance. In conceptualizing social  
33  
34 emotions in research on contestation of a profession's jurisdictional control in this way, our  
35  
36 model goes beyond studies that focus on a single emotion (e.g., Gill & Burrow, 2018); the  
37  
38 framing efforts of one collective actor (e.g., Moisander et al., 2016); and either emotional  
39  
40 evocation (e.g., Goodrick et al., 2020) or emotional resonance (e.g., Giorgi, 2017). By  
41  
42 integrating these components into a single model, our integrative approach to social emotions  
43  
44 makes an important contribution because it allows us to shed light on the origins and critical role  
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46 of ambivalence in jurisdictional contestation of professions by client-led social movements.  
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**Modes of participation in jurisdictional contestation**

We propose five distinct modes of client participation in jurisdictional contestation and theorize how clients come to enact them. Our model explains how ambivalence emerges and plays out in such episodes. Ambivalence is represented to varying degrees and in specific ways in the modes we call opposition, accommodation, and acquiescence. These modes foreground the role of ambivalence in creating conditions for the reconciliation of diverging institutional projects and negotiation of mutually satisfying arrangements despite divisions.

Also, while we have presented these modes separately from one another, empirical research indicates that they coexist and overlap in diverse ways across multiple clients in episodes of jurisdictional contestation. Indeed, empirical studies of social movements disrupting professions have vividly described the tensions between the segments of clientele who enact radical (i.e., escape and opposition), reformist (i.e., accommodation), and conservative (i.e., acquiescence and stewardship) modes of participation (cf. Gould, 2009; Morrison, 2005; Bayer, 1987). But these studies also showed synergies playing out between segments. Especially, many studies observe that the actions of clients enacting the escape and opposition modes created pressures that forced incumbents to yield, partially or entirely, to the aspirations for recognition and inclusion pursued by clients enacting accommodation (cf. Campbell, 2011; Maguire et al., 2004). Our typology of modes enables an analysis of synergies and tensions between segments of a clientele led by different emotional experiences and pursuing different aspirations. It brings together insights from studies of radical contestation (cf. Jasper, 2011; Taylor, 2000) with those from studies of reformist collaboration (cf. DeCelles et al., 2019; Scully & Segal, 2002) to highlight how multiple clients may participate differently in a single episode of jurisdictional contestation by enacting a mode that corresponds to their emotional experience.



By conceptualizing these modes of participation of individual clients in collective action, our model suggests that embracing clients' dissidence and ambivalence towards the profession can help professionals to engage clientele segments in constructive ways that mitigate tensions and tap into synergies. In this way, professionals can channel energy and innovation emerging from clients' experiential knowledge to align professional practices with client needs. Our model therefore responds to growing practical interest across professionalized work domains, and especially in healthcare, for including clients in collaborative design, governance, delivery of, and research on, professional services (Mayo et al., 2021; Boivin et al., 2018; Heaphy, 2017; Fayard et al., 2017). Similar to how professionals incorporate managerial knowledge into their work to defend against jurisdictional contestation by managers (Waring & Currie, 2009), the incorporation of elements of clients' experiential knowledge into professionals' work could mitigate contestation while improving services to clients and shoring up trust in professions.

### **Limitations and future research**

Our model begins with a pre-existing social movement contesting a profession's jurisdictional control and trying to enroll the profession's clients into contestation. We do not explain the emergence of the social movement. However, we do explain its expansion as more individual clients come to enact the escape, opposition, and accommodation modes; as well as its stasis, as clients enact acquiescence (although this truce may be temporary given that the critical consciousness of these clients has been raised). We also suggest conditions of possibility for the emergence of a client-based counter-movement when clients enact stewardship. Future research could focus on the process of emergence of a social movement contesting a profession's jurisdictional control. Such research could inform, on one hand, profession leaders about how to

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foresee and prevent the emergence of such a movement; and, on the other hand, activists about how to organize the emergence of a movement and mobilize adherents.

Also, we treat all social movements the same—as if their framing efforts were always in support of radical contestation of a profession’s jurisdictional control. Of course, empirically, social movements that disrupt professions, and distinct segments within these movements, can pursue a variety of projects, with intended effects on a profession’s jurisdictional control that can range from transformation to reform. Thus, whereas we explain the prevalence of a mode with reference to the resonance of a particular configuration of emotions in individual clients exposed to a movement-profession framing contest, other explanations could focus on the specific content of frames promoted by movements, and how they evolve over time. For instance, future research could compare the framing efforts of more revolutionary movements disrupting a profession with those of more reformist ones to identify differences in the emotional dynamics and enactment of different modes of client participation. Or it could focus on a single movement to explore the tensions and synergies between segments engaging in different framing efforts. Such research could inform both professionals and activists on how to channel tensions and maximize synergies in ways that result in improved alignment of professional services with client needs.

***Beyond the model’s boundaries***

What is specific to relations between clients and professionals, and therefore to the emotional dynamics of jurisdictional contestation, in health-related work domain? How and to what extent might our model apply to other professional work domains? Because “[m]edical practice intrudes into areas of social taboo relating to personal privacy and bodily functions, as well as areas of culturally defined ritual significance such as birth and death” (Johnson, 1972: 43), health-related professional services often stimulate morally tinged emotions (Wright et al.,

2017; Haidt, 2003). This observation suggests that our model may illuminate other contexts where the jurisdictional control of professions reaches deep into the intimacy of clients—into the spiritual, intellectual, or biological confines of their selves—in ways that shape clients' identity. Beyond health-related professions, prototypical cases of such arrangements may be found in education, law-and-order, and religion work domains.

For example, in the education domain, “homeschooling” parent movements organize peer-driven communities and learning centers to address their children’s needs outside of established professional arrangements, binding around their shared pride in educational self-reliance (Neuman & Guterman, 2017). This example illustrates the *escape* mode of participation. Angry protests by Black Lives Matter activists after the acquittal of George Zimmerman (Cullors & Burke, 2018) express the outrage of a clientele segment at what its members experience as morally flawed abuses of law-and-order professionals’ jurisdictional control, demanding transformation of policing practices. It illustrates the *opposition* mode. In the wake of sexual abuse scandals, faithful members of the Catholic Church sought to reconcile felt ambivalence related to their attachment yet mistrust of a clergy that downplayed widespread sex abuses, by advocating for incremental change within the Church’s governance structure (Gutierrez et al., 2010). It illustrates the *accommodation* mode. Future research could focus on episodes of jurisdictional contestation in domains of professional work unrelated to health, yet in which a social movement is also positioned as challenger, to investigate the particular ways in which movement and profession seek to evoke emotions and how the resonance of their framing efforts shapes client participation. Such research could help validate to what extent our model’s insights can illuminate other empirical contexts and identify how social emotion dynamics of client participation vary across domains.

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As concerns health-related work domains, future research could draw on insights from our model but invert the challenger-incumbent relation characterizing our treatment of movement-profession dynamics by exploring the emotional dynamics underpinning the professionalization of mutual aid arrangements, a pervasive phenomenon seen by social critics as corrosive of the social fabric (see Scott, 2014; Hochschild, 2012; McKnight, 1995; Larson, 1977). For example, Zilber (2002) studied a rape crisis center in Israel which was founded by feminist activists under mutual aid principles. As therapy-oriented staff began populating the center, mutual aid principles were backgrounded and replaced by professional principles, as Zilber (2002: 248) explains: “according to a feminist view, victims were supposedly treated as equals. With the institutional change, they became ‘patients’, or in a more politically correct phrase, ‘clients’; in any case, they became pathologized, bundles of symptoms.” In this process, which can be understood as a jurisdictional contestation process in reverse, the expert knowledge of therapists replaced the experiential knowledge of feminist activists as the dominant epistemic basis of social arrangements at the clinic. Focusing on the social emotions shaping this process may expand our understanding of how mutual aid arrangements become professionalized; and insights contained in our model may offer a helpful starting point for such an inquiry.

**Concluding remarks**

In this paper, we break new conceptual ground by theorizing how emotions shape the participation of health-related professions’ clients in episodes of jurisdictional contestation. By offering an integrated and multi-level understanding of these processes, our model informs not only research but also the practice of both health-related professionals and clients seeking to improve services by leveraging clients’ experiential knowledge to alter professionals’ work. In

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3 short, we hope that our paper makes a forceful case for engaging clients rather than dismissing  
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5 them, in both theory and practice.  
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## TABLES AND FIGURES

Table 1—Framing efforts and the emotions they seek to evoke

| <i>Collective actor</i>                              | <i>Intent of framing efforts</i>                                                 | <i>Content of frames</i>                                                               | <i>Emotion sought to be evoked and its object</i>                     |
|------------------------------------------------------|----------------------------------------------------------------------------------|----------------------------------------------------------------------------------------|-----------------------------------------------------------------------|
| <b>Social movement (disruptive collective actor)</b> | Strengthening the client's trust in the social movement's experiential knowledge | Experiential knowledge is relevant, safe, and/or reliable                              | Pride in supporting an alternative, peer-driven institutional project |
|                                                      | Gaining the client's attachment to the social movement as a collective actor     | Deviance from the profession's prescriptions is a righteous imperative                 | Anger at the profession for breaching its duty                        |
|                                                      | Causing the client's detachment from the profession as a collective actor        | Compliance with the profession's prescriptions is morally flawed and/or self-degrading | Shame of complying with the profession's prescriptions                |
|                                                      | Undermining the client's trust in the profession's expert knowledge              | Expert knowledge is irrelevant, dangerous, and/or unreliable                           | Fear of relying on established arrangements                           |
| <b>Profession (defensive collective actor)</b>       | Strengthening the client's trust in the profession's expert knowledge            | Expert knowledge is relevant, safe, and/or reliable                                    | Pride in supporting established arrangements                          |
|                                                      | Gaining the client's attachment to the profession as a collective actor          | Compliance with the profession's prescriptions is a righteous imperative               | Anger at movement for jeopardizing a proper response to their needs   |
|                                                      | Causing the client's detachment from the social movement as a collective actor   | Deviance from the profession's prescriptions is morally flawed and/or self-degrading   | Shame of deviating from the profession's prescriptions                |
|                                                      | Undermining the client's trust in the social movement's experiential knowledge   | Experiential knowledge is irrelevant, dangerous and/or unreliable                      | Fear of relying on an alternative, peer-driven institutional project  |

Table 2—Successful framing efforts and modes of participation in jurisdictional contestation

| <i>Framing efforts that evoke emotions successfully</i>                                                                   | <i>Emotional resonance</i>                                                                                                                                                                                                                                                                                                                                                                                          | <i>Mode of participation in jurisdictional contestation</i>                                     |
|---------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------|
| Only the social movement’s framing efforts are successful                                                                 | Pride in supporting alternative project (greater degree), anger at profession, shame of complying with profession’s prescriptions and fear of relying on established arrangements (lesser degrees)                                                                                                                                                                                                                  | <b>Escape:</b><br>Seeking to withdraw from established arrangements for peer-driven alternative |
| Both framing efforts are successful to some degree, but the social movement’s framing efforts dominate (some ambivalence) | Anger at profession (greater degree), shame of complying with profession’s prescriptions and pride in supporting alternative project yet fear of relying on it (lesser degrees)                                                                                                                                                                                                                                     | <b>Opposition:</b><br>Seeking to radically transform established arrangements                   |
| Both framing efforts are successful to a comparable degree (full-fledged ambivalence)                                     | Pride in supporting alternative project but shame of deviating from profession’s prescriptions (greater degrees) and anger at profession yet fear of relying on alternative project (lesser degrees) <i>or</i> anger at profession but fear of relying on alternative project (greater degrees) and pride in supporting alternative project yet shame of deviating from profession’s prescriptions (lesser degrees) | <b>Accommodation:</b><br>Seeking to incrementally improve established arrangements              |
| Both framing efforts are successful, but the profession’s framing efforts dominate (some ambivalence)                     | Shame of deviating from profession’s prescriptions (greater degree), anger at social movement and fear of supporting alternative project yet pride in supporting it (lesser degrees)                                                                                                                                                                                                                                | <b>Acquiescence:</b><br>Seeking to passively accept established arrangements                    |
| Only the profession’s framing efforts are successful                                                                      | Fear of relying on alternative project (greater degree), shame of deviating from profession’s prescriptions, anger at social movement and pride in supporting established arrangements (lesser degrees)                                                                                                                                                                                                             | <b>Stewardship:</b><br>Seeking to actively reinforce established arrangements                   |



Figure 1—Emotional resonance of a movement/profession framing contest: an overview

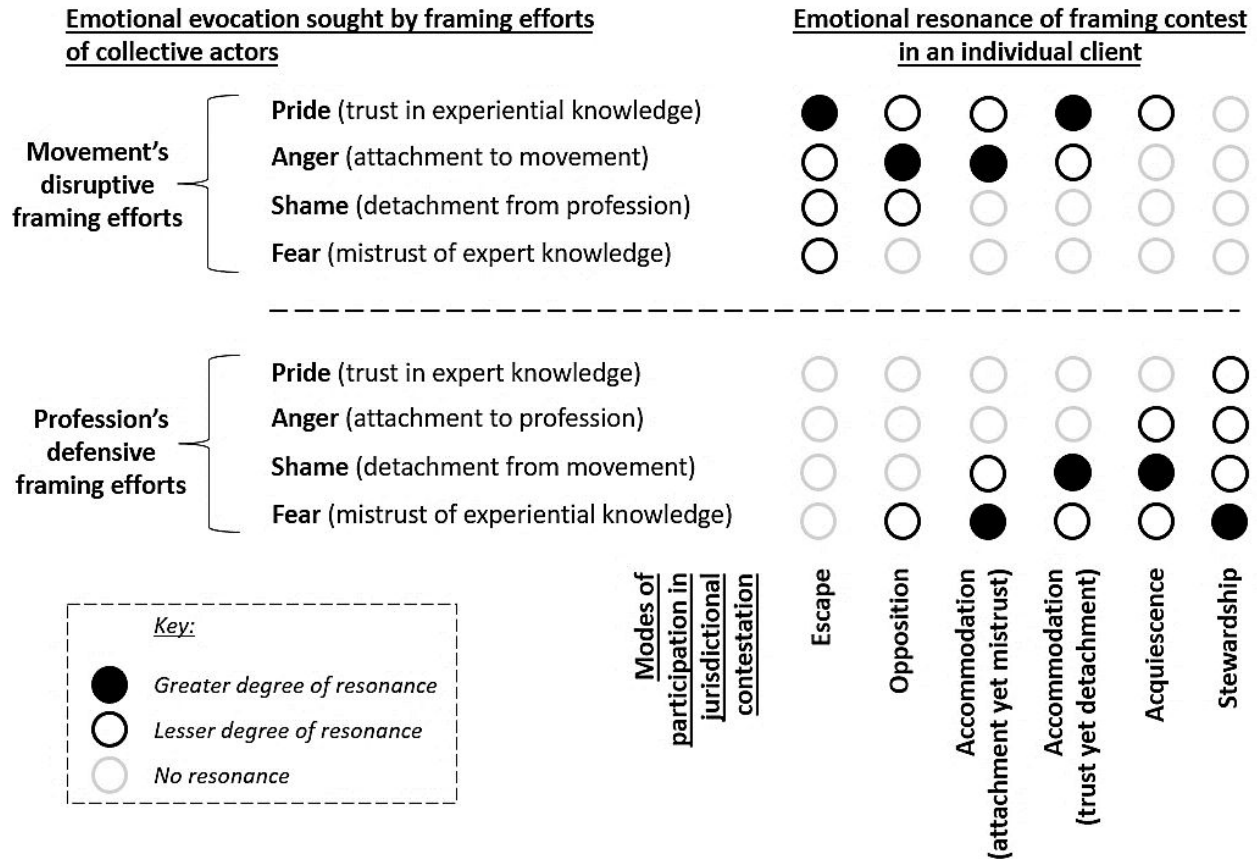


Figure 2—Emotional resonance leading a client to enact the ‘escape’ mode of participation

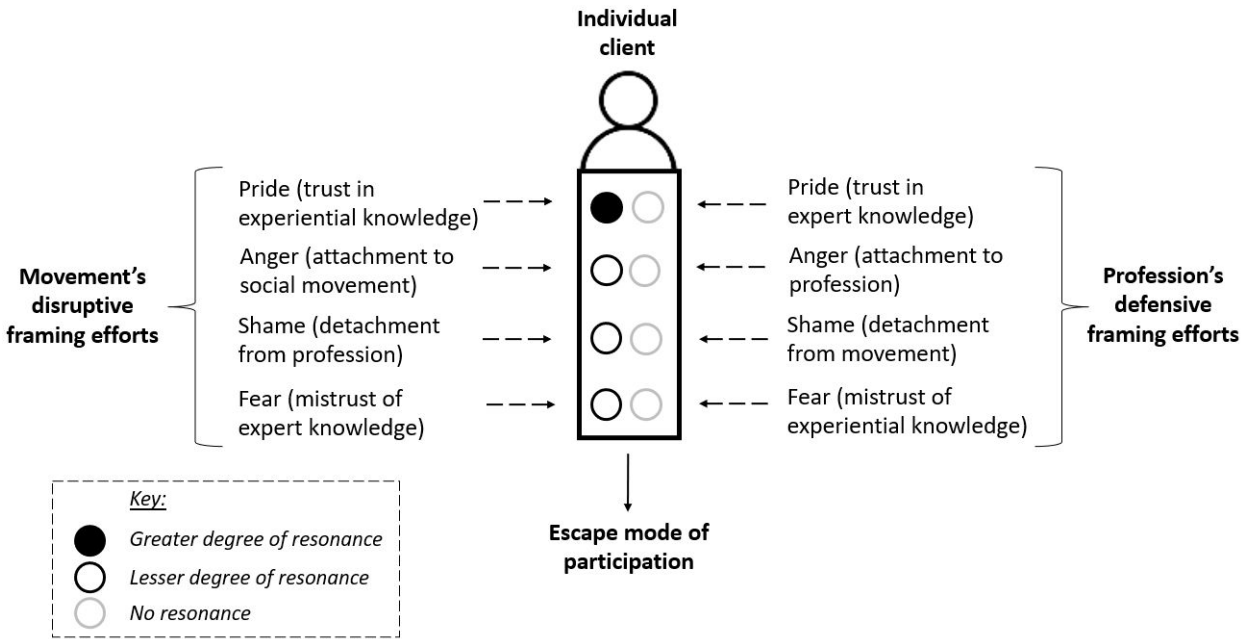


Figure 3—Emotional resonance leading a client to enact the ‘opposition’ mode of participation

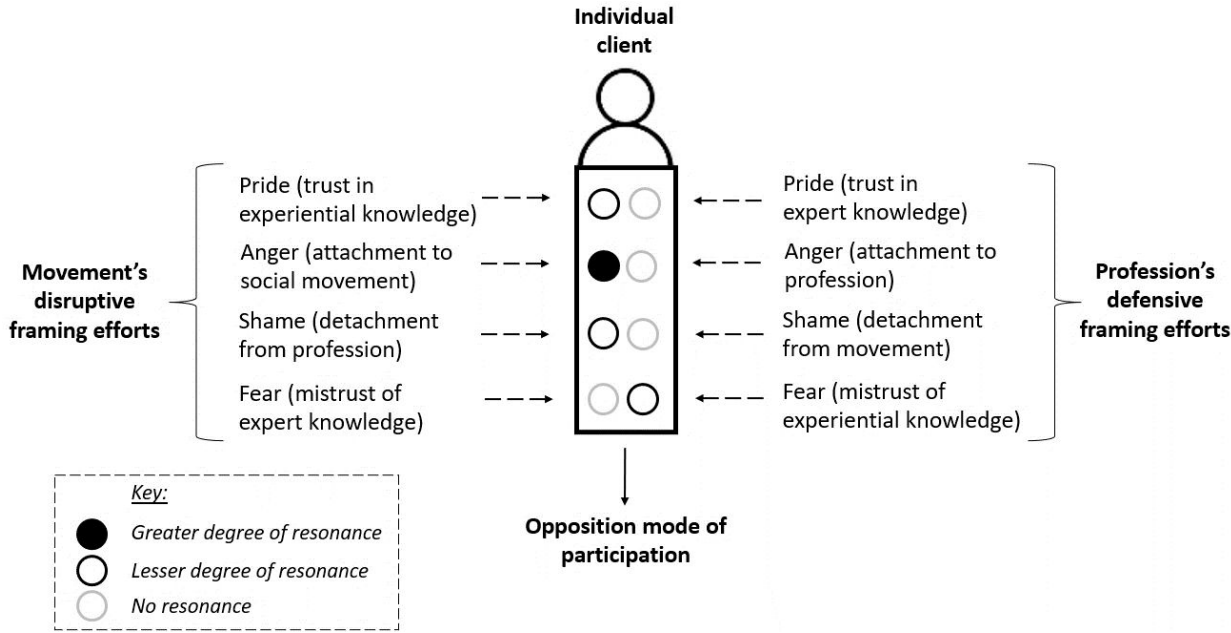


Figure 4a—Emotional resonance leading a client to enact the ‘accommodation’ mode of participation (with ‘attachment yet mistrust’ form of ambivalence)

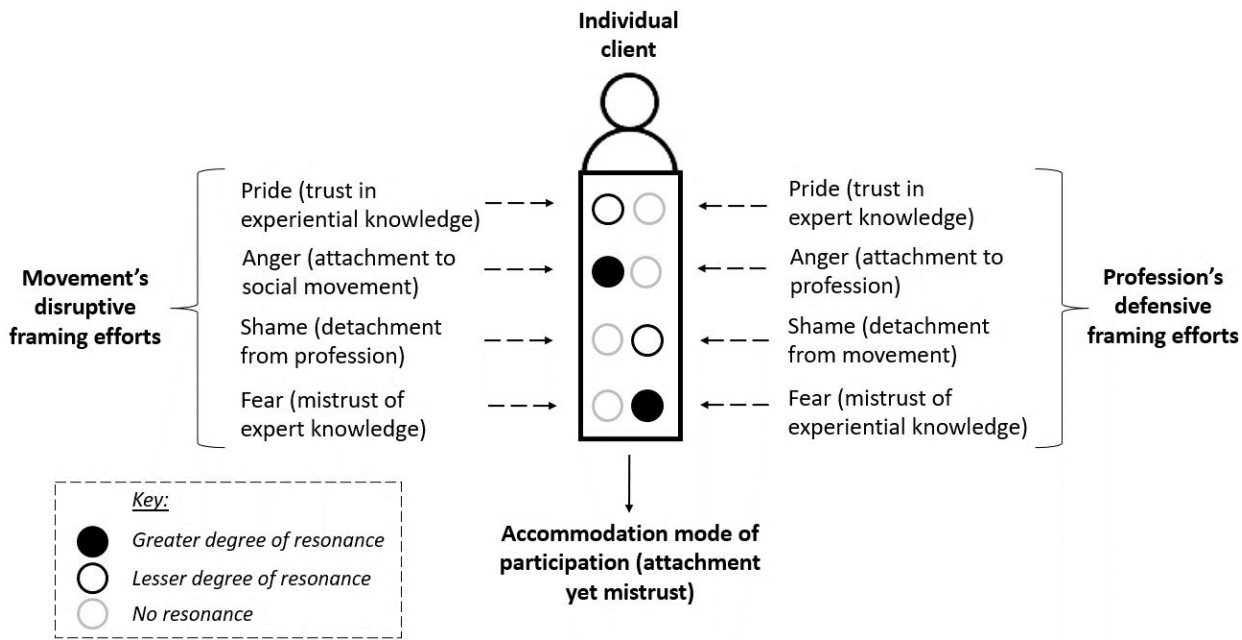


Figure 4b—Emotional resonance leading a client to enact the ‘accommodation’ mode of participation (with ‘trust yet detachment’ form of ambivalence)

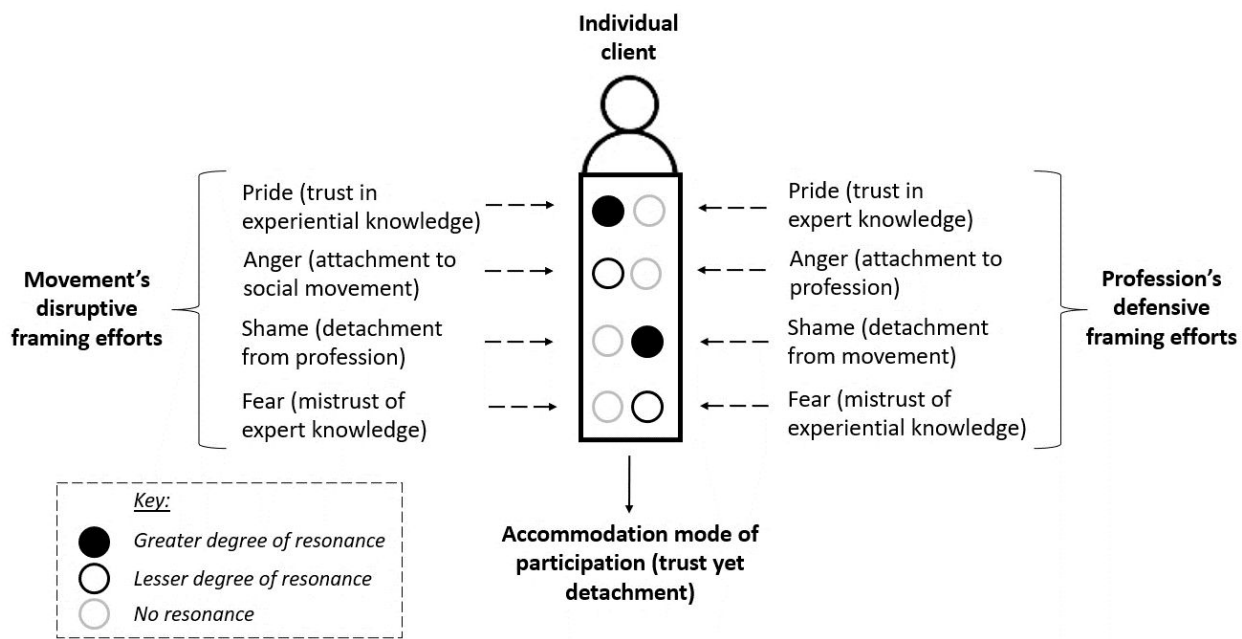


Figure 5—Emotional resonance leading a client to enact the ‘acquiescence’ mode of participation

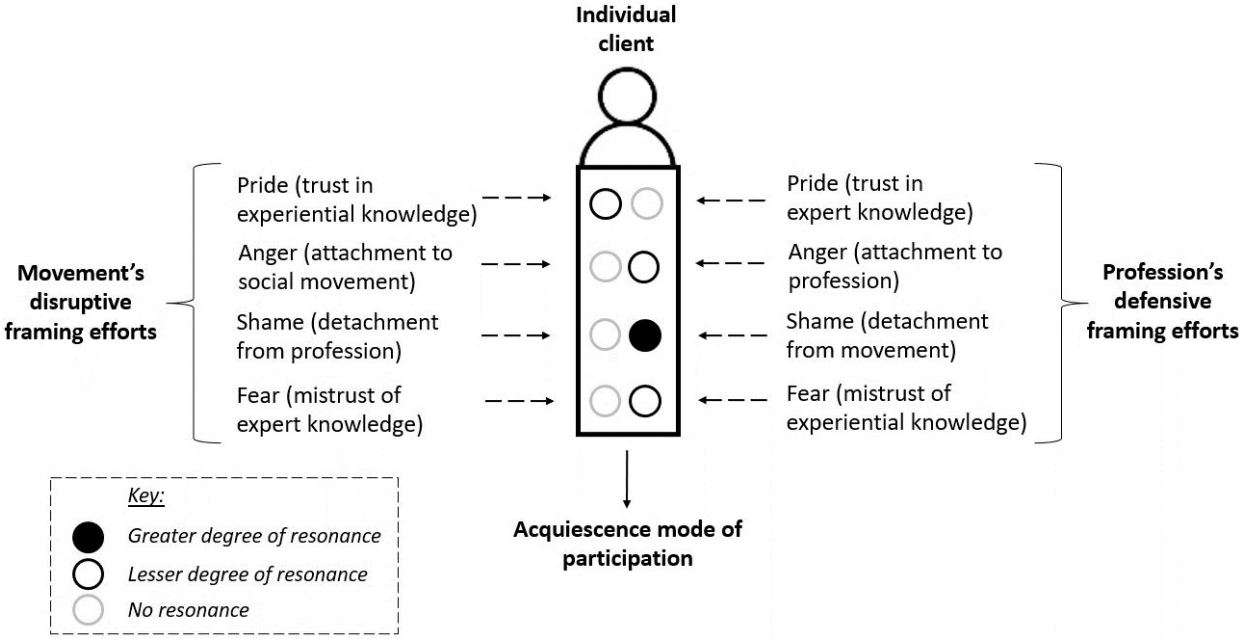
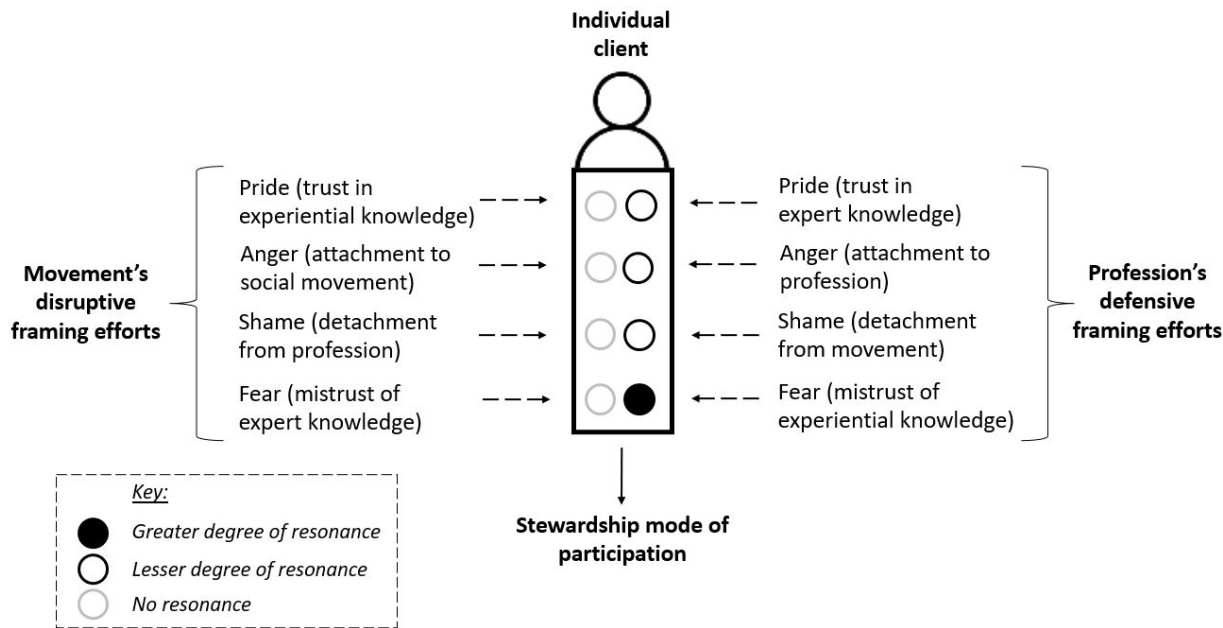


Figure 6—Emotional resonance leading a client to enact the ‘stewardship’ mode of participation



## Short bios of authors

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